



Exam requested by:
ARIEL PEDOEEM DO
377 BROADWAY
NEWBURGH NY [REDACTED]

SITE PERFORMED: LHR NEWBURGH

Patient: [REDACTED]
Date of Birth: [REDACTED]
Phone: (704) 349-0802
[REDACTED]
Date of Exam: [REDACTED]

EXAM: DIGITAL BILATERAL DIAGNOSTIC MAMMOGRAM AND TOMOSYNTHESIS AND BREAST ULTRASOUND

HISTORY: The patient [REDACTED] since breast biopsy- without palpable mass. Family history of breast cancer maternal aunt.

Age: 46 years old.

CLINICAL BREAST EXAMINATION: The patient [REDACTED] clinical breast exam.

COMPARISON: The present examination is compared to prior imaging studies dating back to 2022.

MAMMOGRAM:

TECHNIQUE: Low-dose full-field digital breast tomosynthesis examination was performed with 2D and 3D acquisitions. The following views were obtained digitally: bilateral craniocaudal, bilateral mediolateral oblique. Computer-aided detection was applied.

FINDINGS:

BREAST COMPOSITION: There are scattered areas of fibroglandular density.

Stable post biopsy changes are identified in the left breast. An area of density is identified in the upper inner left breast, 9 cm from the nipple, adjacent to a dense calcification shows post biopsy changes and corresponds to the lateral calcifications biopsied on [REDACTED]. This may correspond to the "enhancing mass in the upper outer mid to posterior left breast at approximately 2:30 o'clock 11 cm from nipple" identified on breast MRI [REDACTED].

The remainder of the left as well as the right breast are unremarkable and no [REDACTED] distortion, or significant calcifications are detected.

BREAST ULTRASOUND:

TECHNIQUE: Complete left breast ultrasound, with evaluation of the four quadrants, retroareolar region, and axilla, was performed.

FINDINGS:

Left breast:

2:30 11 cm from nipple-second look evaluation of breast MRI finding [REDACTED] iso to mildly hypoechoic nodule versus focus of prominent breast parenchyma-1.4 x 1.4 x 0.4 cm. This does not correlate with the breast MRI finding which

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demonstrated an area of enhancement measuring 2.5 cm AP x 0.9 cm W
9 o'clock 8 cm from nipple cystic collection-0.6 x 0.6 x 0.5 cm. This corresponds to the region of the biopsy and therefore likely represents post biopsy changes.
9 o'clock 8 cm from nipple hypoechoic nodule-0.3 x 0.8 x 0.9 cm. New-6 month short interval follow-up warranted.

IMPRESSION:

1. Post biopsy changes left breast as described and discussed above.
2. 1.4 cm nodule identified in the left breast 2:30 11 cm from nipple, however, this does not correlate with the breast MRI 2.5 cm focus of enhancement. The recommendation for MRI guided core biopsy of this area remains unchanged.

FOLLOW-UP: Follow-up imaging in 6 months.

Targeted left breast ultrasound 9 o'clock 8 cm from nipple.

MRI biopsy-enhancing mass identified on [REDACTED]

ASSESSMENT: BI-RADS Category 3: Probably benign.

Left breast 9 o'clock 8 cm from nipple hypoechoic nodule

The patient [REDACTED] situ-status post stereotactic [REDACTED]

This examination should not preclude the clinical evaluation of a suspicious palpable abnormality.

As per the FDA requirements, a lay letter has been generated and sent to your patient [REDACTED]
recommendations of this breast imaging study. We have entered your patient [REDACTED]
them when they are due for their next breast imaging exam.

Thank you for the opportunity to participate in the care of this patient.

STEVEN G REICHARD DO - *Electronically Signed:* [REDACTED] 3:53 PM

Physician [REDACTED]

FDA Site Name: [REDACTED]

320 Robinson Ave, Newburgh, NY [REDACTED]

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